

Loramide Capsule 2mg

Ingredient(s):

Each capsule contains:

Loperamide HCl 2mg

Pharmacodynamics:

1. Loperamide slows intestinal motility through a direct effect on the nerve endings and/or intraneural ganglia of the intestinal wall. It is generally believed to act by interfering with the cholinergic and non-cholinergic mechanisms involved in the peristaltic reflex, decreasing the activity of circular and longitudinal muscles in the intestinal wall. However, some data indicate that it may act by enhancing contractions of intestinal circular musculature, thus increasing segmentation and retarding forward motion through the intestine.
2. Loperamide prolongs the transit time of intestinal contents and therefore reduces fecal volume, increases fecal viscosity and bulk density, and diminishes loss of fluid and electrolytes.

Pharmacokinetics:

Peak plasma concentrations of Loperamide occur about 4-5 hours following administration of the capsules. The elimination half-life of Loperamide in healthy adults is 10.8 hours (range 9.1 – 14.4 hrs). It is 95% bound to plasma proteins. Less than 2% of a 4mg dose is excreted in the urine and 30% is excreted in the feces as intact drug. It has been shown to undergo enterohepatic circulation in animals.

Indications:

Indicated for the control and symptomatic relief of acute non-specific diarrhea and of chronic diarrhea associated with inflammatory bowel disease.

Route of Administration :

Oral.

Dosage and Administrations:

Adults:

Acute diarrhea:

Oral, 4mg after first loose bowel movement, followed by 2mg after each subsequent loose bowel movement.

Chronic diarrhea:

Initial: Oral, 4mg, followed by 2mg after each subsequent loose bowel movement until diarrhea is controlled.

Maintenance: Oral, 4 to 8mg a day in divided daily doses as needed.

Daily dosage should not exceed 16mg.

Children:

Children above 6 years of age: Oral, 2mg twice a day.

Children 8 to 12 years of age: Oral, 2mg three times a day.

Maintenance: Oral, reduced to meet individual requirements; some clinicians recommended one-third to one-half the initial dose.

To be dispensed on physician's prescription.

Contraindications:

1. Loperamide should not be used in the treatment of acute diarrhea associated with organisms that penetrate the intestinal mucosa (eg. toxigenic E. coli, Salmonella, and Shigella) or in patients with pseudomembranous colitis associated with broad-spectrum anti-infectives.
2. This drug is contraindicated in patients with known hypersensitivity to the drug and in patients in whom constipation must be avoided.

Warnings and Precautions:

Loperamide is not recommended for children under 6 years of age. Its use has been associated with fatal episodes of paralytic ileus in infants and young children.

1. In acute diarrhea, if clinical improvement is not observed within 48 hours, the administration of Loperamide should be discontinued.
2. Patients with hepatic dysfunction should be monitored closely for signs of CNS toxicity because of the apparent large first pass biotransformation.
3. Because of the greater variability of response in young children, Loperamide should be used with particular caution in this age group. Dehydration may further influence the variability of response.
4. This medication should not be used in the case of acute dysentery which is characterized by elevated temperatures and blood in stools.
5. If fluid and electrolyte depletion occur in the patients taking Loperamide, administration of appropriate fluid and electrolyte therapy should be considered.
6. Loperamide therapy should be discontinued promptly if abdominal distension occurs or if other troublesome symptoms develop in patients with acute ulcerative colitis.
7. Appropriate fluid and electrolyte therapy should be given to protect against dehydration in all cases of diarrhea. Oral rehydration therapy – which is the use of appropriate fluids including oral rehydration salts – remains the most effective treatment for dehydration due to diarrhea. The intake of as much of these fluids as possible is therefore imperative.
8. Drug – induced inhibition of peristalsis may result in fluid retention in the intestine, which may aggravate and mask dehydration and depletion of

electrolytes, especially in young children. If severe dehydration or electrolyte imbalance is present, Loperamide should be withheld until appropriate corrective therapy has been initiated.

9. The use of higher than the recommended doses for control of the diarrhea may cause abnormal heart rhythms and serious cardiac events leading to death. However, in adult patients receiving the recommended dosage of Loperamide, cases of syncope and ventricular tachycardia have been reported. Some of these patients were taking other drugs or had other risk factors that may have increased their risk of cardiac adverse reactions.
10. Abuse and misuse of Loperamide, as an opioid substitute, have been described in individuals with opioid addiction (*see Symptoms and Treatment of Overdose*).

Pregnancy and Lactation:

Although there are no indications that Loperamide HCl possesses teratogenic or embryotoxic properties, the anticipated therapeutic benefits should be weighed against potential hazards before Loperamide HCl is given during pregnancy, especially during the 1st trimester.

Small amounts of loperamide HCl may appear in human breast milk. Therefore, Loperamide HCl is not recommended during breastfeeding.

Side Effect / Adverse Reactions:

Loperamide is generally well tolerated. However, abdominal pain, distension or discomfort, constipation, drowsiness, dizziness, fatigue, dry mouth, nausea and vomiting, and epigastric pain may occur. Hypersensitivity reactions including rash have been reported. Children may be more sensitive to adverse CNS effects of the drug than adults.

Gastrointestinal disorders

Frequency 'not known': Acute pancreatitis

Post-marketing Experience

Cardiac Disorders: QT/ QTc interval prolongation, Torsades de Pointes, other ventricular arrhythmias, cardiac arrest, syncope, and death (*see Warnings and Precautions*).

Interactions with Other Medicaments:

Concurrent use of Loperamide with an opioid analgesic may increase the risk of severe constipation.

Symptoms and Treatment For Overdosage, and Antidotes:

Symptoms: Constipation, gastrointestinal irritation & CNS depression.

Treatment: A slurry of activated charcoal administered promptly after ingestion of Loperamide HCl can reduce the amount of drug which is absorbed into the systemic circulation. If vomiting occurs spontaneously upon ingestion, a slurry of 100gms of activated charcoal should be administered orally as soon as fluids can be retained.

If vomiting has not occurred, gastric lavage should be performed followed by administration of 100gms of the activated charcoal slurry through the gastric tube. In the event of overdosage, patient should be monitored for signs of CNS depression for at least 24 hours. Children maybe more sensitive to CNS effects than adults. If CNS depression is observed, naloxone may be administered. If responsive to naloxone, vital signs must be monitored carefully for recurrence of symptoms of drug overdose for at least 24 hours after the last dose of naloxone.

In individuals who have intentionally ingested overdoses (reported in doses from 40mg up to 792mg per day) of Loperamide HCl, prolongation of the QT/QTc interval, Torsades de Pointes, other ventricular arrhythmias and cardiac arrest, have been observed (*see Warnings and Precautions*). Fatal cases have also been reported.

Abuse, misuse and/or overdose with excessively large doses of loperamide, may unmask Brugada syndrome.

Shelf-life:

3 years from the date of manufacture.

Storage Conditions:

Keep in a tight container. Store at temperature below 30°C. Protect from light and moisture.

Product Description:

A #4 dark green cap and grey body hard gelatin capsule, containing white powder.

Dosage Form and Packaging Available:

Capsule in :

Plastic bottle of 1000's and 1500's (for export only)

Plastic bottle of 100's

Blister packing of 10's x 10 and 10's x 50



Manufacturer and Product Registration Holder:
Y.S.P. INDUSTRIES (M) SDN. BHD. (199001001034)
Lot 3, 5, 7, 12 & 14, Jalan P/7, Section 13,
Kawasan Perindustrian Bandar Baru Bangi,
43000 Kajang, Selangor, Malaysia.
Ordering Line: 1 800 88 3027
Product Info: 1 800 88 3679